



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : سميحه عبدالسميع السيد ابراهيم

العنوان : القليوبية

الحي / المدينة : شبرا الخيمة

المحافظة : القليوبية

تاريخ تسجيل الإستمارة : 30-09-2026

رقم البطاقة : 24804051701243 رقم التليفون : 01118100188

العمر (تاريخ الميلاد) : 05-04-1948 المهنة : بدون عمل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة

قسم العلاج :
الأورام

[11129]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /د. /د. /د. /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: سميحة عبد السميع السيد ابراهيم National ID: 24804051701243	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة عضو النخبة



سميحه
عبدالمسيح السيد ابراهيم
٢٥٠ ح مئسوش الانتاج - عزبه صقل
شبرا الخيمة اول - القليوبية

٢٤٨ . ٤ . ٥ ١٧ . ١٢ ٤٣

KM4723189

٢٠٢٤/٠٣ ٢٤٨٠٠٥٠١٧٠١٢٤٣

رنيه منزل
انثى مسلمة
المنطقة مصرية حتى ٢٠٣١/٠٣/١٠






أفد
٨ - ١٧



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الست المصرية
صحة مصر

08/16/2026

MDT FORM

Name:	Samiha Abd Elsamie El Sayed Iprahim //	MRN:	1702
Age:	78 Y	ID:	24804051701243
Menopausal Sta		Phone:	01222369411
NRS SCORE:		Marital Status:	WIDOW
BMI:		Family History:	
CO Morbidity:	dm ,HTN	Reason Of MDT:	POST OP

Clinical Data:

R

L

BX: IDC G11
ER: 8/8 PR: 8/8 HER2: 0 KI67: 30

Sonomammography:

Mets Workup:

FREE

History:

PTN was T4 N3 M0 POST NACT underwent LT MRM

POP:

Residual viable IDC G11 Low grade DCIS

FREE margins

LN :6/7 WITH extranodal extension

pT2, ypN2

Recommendations:

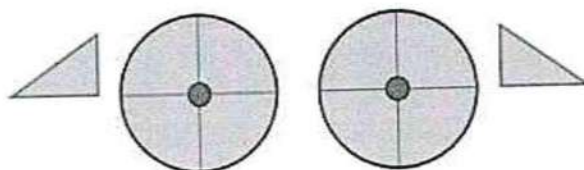
for possibility of adjuvant ttt

MDT Outcome & Plan

FOR port

adj femara 7 years

zometa every 6 ms after dental massessment



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د حنان عزت	د أحمد نجيب				

Patient Name	سميحة عبد السميع السيد	Case No.	341961
Sex/Age	Female 78 Y	Patient No.	24762
Registered Date	14/7/2026 11:23:39 AM	Referred By Dr.	Surgery
Validation Date	13/8/2026 12:10:54 PM	Branch	Gustave Roussy
Reporting Date	13/8/2026 12:11:21 PM	Pathology No.	2217

Pathology Report

Clinical Data: Left breast cancer, post-NACT.

Nature of the specimen: Left modified radical mastectomy.

Gross Examination:

- * Left modified radical mastectomy specimen measured 25x15x10 cm, with covering skin ellipse measured 20x15 cm. Nipple & areola looked unremarkable grossly.
- * Serialing showed an ill-defined firm mass measured 2.8x1.8x1.5 cm, surrounding by rubbery grey, white area & located 5 cm from the deep margin.
- * Axillary fat entangling 7 lymph nodes, the largest measured 1.0x1.0 cm.
- * Block designation:
- * 16 Blocks:
- 1,2- G/W area., 3,4- Mass., 5:10- G/W area., 11-Deep margin., 12- N&A., 13:16- ALNs.
- Gross was done by; Dr. Aya Ezzat.

Microscopic examination:

- **Histological type:** Residual viable invasive duct carcinoma.
- **Tumor size:** 2.8x1.8 cm
- **Tumor Cellularity:** 50%.
- **Grade:** 2 (differentiation 3, nuclear pleomorphism 2, mitosis 1).
- **Ductal carcinoma in situ:** 5% Low grade DCIS (Solid & Cribriform patterns).
- **Angioinvasion:** Present.
- **Tumor infiltrating lymphocytes (TILs):** Mild.
- **Perineural invasion:** Absent.
- **Microcalcification:** Absent.
- **Nipple and areola:** Free.
- **Lymph node involvement:** Positive viable metastatic deposits in six out of seven lymph nodes (6/7) (The metastatic focus measuring 0.6 cm, with extra-nodal extension).
- **Resection Margins:** Free deep margin.
- **Other pathological findings:** In abreast with fibrocystic changes.
- **Histologic response to therapy:** Pathological partial response.

Diagnosis:



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Gustave Roussy International-Egypt- Pathology Lab
جوستاف رومسي (القاهرة) - مصر - معهد الباثولوجي



Patient Name	سميحة عبد السميع السيد	Case No.	341961
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Reporting Date	13/8/2026 12:11:21 PM	Pathology No.	2217

Left Breast Cancer (Post-NACT); Left Modified Radical Mastectomy:

- Residual viable invasive duct carcinoma, Grade 2 (Cellularity 50%) (Actual tumor size 2.8x1.8 cm).
- 5% Low grade ductal carcinoma in situ (Solid& Cribriform patterns).
- Nipple and areola: Free.
- Deep margin: Free.

- Axillary lymph nodes: Positive viable metastatic deposits in six out of seven lymph nodes (6/7) (The metastatic focus measuring 0.6 cm, with extra-nodal extension).

Stage: ypT2, ypN2.

RCB: Tumor cellularity 50%, RCB= 3.754, RCB class III.

Signature

Dr. Walaa Ibrahim Ghonaim, MD.



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Patient Name	: الأستاذة / منيرة عبدالمعطي السيد ابراهيم	Request No.	: 13008466
Patient No.	: 248040501701243	Request Date	: 20/11/2024
Path No.	: 4499/24	Verification Date	: 07/12/2024
Sex	: Female	Financial Category	: أورام دار السلام - مبادئ
Age	: 76 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.
Nature of Specimen: Tru-cut biopsy.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 34% nuclear reaction of tumor cells.

Conclusion:

Left breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II.
IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67: Positive in 34% of tumor cells.

Comment:

- * Scoring system: "ER, PR"

Verified by :

Dr. Heba Hasan

Printing Date : 07/12/2024 10:37:48

Page 1 of 2

Printed By : marwa mohamed youss



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ame: Mrs. samiha abdelsamele elsayed
Date: 20-12-2025

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: pathologically proven left breast carcinoma , received chemotherapy and hormonal treatment

Findings:

Right breast.

- Scattered fibroglandular density (ACR b)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- Ultrasound demonstrated homogenous fatty parenchyma
- no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- No significant axillary lymph nodes

Left breast.

- Scattered fibroglandular density (ACR b)
- An Irregular speculated dense mass is seen in the upper central region with detected internal microcalcifications within
- Thickened skin and course trabeculae
- Ultrasound demonstrated an Irregular spiculated hypoechoic mass at 12 o'clock zone a measuring 17 x 18 mm with detected internal vascularity within
- Non specific axillary lymph nodes

Conclusion:-

Right breast (BIRADS 1): normal

Left breast (BIRADS 6): pathologically proven breast carcinoma

Dr farah elshobaky



Dr. Islam Radiology Center

مركز الدكتور إسلام الشاذلي للأشعة

دكتوراه الأشعة التشخيصية
إستشاري ومدرس الأشعة
التشخيصية بكلية الطب

Name: الاسم: سميحة عبد السميع
Date: 11-11-2024
Ref: ا.ر: عبد الجواد فؤاد

Report

BILATERAL SOFT TISSUE SONO-MAMMOGRAPHY

- A spiculated ill-defined, soft tissue dense opacity is noted at the upper half of the left breast with distorted breast parenchyma. No detectable micro-calcifications.
- Normal appearance of the right breast parenchyma with predominantly fatty breast
- Enlarged left axillary nodes.

*COMPLEMENTARY ULTRASOUND:

- There is an ill-defined spiculated heterogeneous hypoechoic mass lesion at the left breast upper half having (12 O'clock) periareolar location. Measuring 1.6×1.4 cm in maximum dimensions.
- A well defined, cystic, unilocular breast lesion with turbid fluid content is seen at upper outer quadrant of the left breast at (1 O'clock). It measures (0.7×0.5 cm) in maximum dimensions. No detected mural wall thickening or calcification suggesting galactoceles.
- Multiple left axillary, hypoechoic Metastatic lymphadenopathy are also seen (the largest measuring 1.6×1.3 cm in maximum dimensions)

*CONCLUSION

- LEFT BREAST MASS LESION (BI-RADS - 5)
- LEFT BREAST GALACTOCELE.

Bi-Rads Classification For Breast Disorders

- | | |
|---|--|
| 0 | - Need Additional Imaging Evaluation
or Prior Mammograms For Comparison |
| 1 | = Negative
There is nothing to comment on |
| 2 | = Benign Finding |
| 3 | = Probably Benign Finding (<2% malignant)
Initial Short-Interval Follow-Up Suggested |
| 4 | = Suspicious Abnormality (2 - 95% malignant)
Biopsy Should Be Considered |
| 5 | = Highly Suggestive of Malignancy (>95% malignant)
Appropriate Action Should Be Taken |
| 6 | = Known Biopsy - Proven Malignancy |

Thanks
Dr. ISLAM EL SHAZLY (M.D.)

*Maadi Military Medical Compound
Oncology and Hematology Hospital
Nuclear Medicine Department*

Patient Name: Samiha Abdel-Samea El-Sayed
Date of Scan: 18/12/2024
Patient ID: 1_429575

^{99m}Tc-MDP SKELETAL SCINTIGRAPHY

History

- 77-year-old lady, with history of left breast cancer, referred for metastatic workup.

Technique

- Anterior and posterior whole body scan were acquired 2 hours post injection of the tracer.

Findings:

- Mild increased tracer uptake is seen at DVI9 and DVI2; likely degenerative in nature, for correlation with radiological data.
- Otherwise, no abnormal active or cold bony lesions detected over the surveyed body to account for metastatic bone disease.

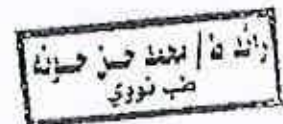
Impression:

- No scintigraphic evidence of metastatic bone disease.

Best regards,

Dr. Ahmed Abdel-Karim

Dr. Mohamed Hassan, MSc



8/16/26, 12:29 PM

code:02 -- Name_A MDT -- Name_E MDT

جولف
روستاق
مركز



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

المستشفى
مركز

08/16/2026

MDT FORM

Name:	Samihla Abd Elsamie El Sayed Iprahim //	MRN:	1702
Age:	78 Y	ID:	24804051701243
Menopausal Sta		Phone:	01222369411
NRS SCORE:		Marital Status:	WIDOW
BMI:		Family History:	
CO Morbidity:	dm ,HTN	Reason Of MDT:	POST OP

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BX:	IDC G11
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R

L

Sonomammography:



Mets Workup:

FREE

History:

PTN was T4 N3 M0 POST NACT underwent LT MRM
POP:
Residual viable IDC G11 Low grade DCIS
FREE margins
LN :6/7 WITH extranodal extension
pT2, ypN2

T N M

Recommendations:

. for possibility of adjuvant ttr

MDT Outcome & Plan

FOR port
adj femara 7 years
zometa every 6 ms after dental massessment

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. هادي عريش	د. هادي عريش				

CH

Patient Name	سيدة عبد السميع السيد	Case No.	341961
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 - * 16 Blocks:
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- Gross was done by; Dr. Aya Ezzat.

Microscopic examination:

- Histological type: Residual viable invasive duct carcinoma.
- Tumor size: 2.8x1.8 cm
- Tumor Cellularity: 50%.
- Grade: 2 (differentiation 3, nuclear pleomorphism 2, mitosis 1).
- Ductal carcinoma in situ: 5% Low grade DCIS (Solid & Cribriform patterns).
- Angioinvasion: Present.
- Tumor infiltrating lymphocytes (TILs): Mild.
- Perineural invasion: Absent.
- Micrometastasis: Absent.
- Nipple and areola: Free.
- Lymph node involvement: Positive viable metastatic deposits in six out of seven lymph nodes (6/7) (The metastatic focus measuring 0.6 cm, with extra-nodal extension).
- Resection Margins: Free deep margin.
- Other pathological findings: In abreast with fibrocystic changes.
- Histologic response to therapy: Pathological partial response.

Diagnosis:



مختبر الأمراض - مصر
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جوستاف روسي للسرطان - مصر (987) - طابق أول - مبنى رقم 4 - القاهرة - مصر

341961|6279|



Gustave Roussy International-Egypt- Pathology Lab
جورستاف رويسى انترناتيونال - مصر مستشفى الجورستافى



Patient Name	سميرة عبد السميع السيد	Case No.	341961
Sex/Age	Female 78 Y	Patient No.	24762
Registered Date	14/7/2026 11:23:39 AM	Referred By Dr.	Surgery
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Left Breast Cancer (Post-NACT); Left Modified Radical Mastectomy:

- Residual viable invasive duct carcinoma, Grade 2 (Cellularity 50%) (Actual tumor size 2.8x1.8 cm).
- 5% Low grade ductal carcinoma in situ (Solid& Cribriform patterns).
- Nipple and areola: Free.
- Deep margin: Free.
- Axillary lymph nodes: Positive viable metastatic deposits in six out of seven lymph nodes (6/7) (The metastatic focus measuring 0.6 cm, with extra-nodal extension).
- Stage: ypT2, ypN2.
- RCB: Tumor cellularity 50%, RCB= 3.754, RCB class III.

Signature

Dr. Walaa Ibrahim Ghonaim, MD.



طريق رقم (4) - الدور الاول - مبنى رقم (987) - الدور الثاني - مبنى القبة - الطابق 5 - مصر
Kottan No. (4) - First Floor-Building No. 987- Nile Corniche - Miter El Qudema - Cairo, Egypt



SCAN & LABS

مركز النيل للأشعة والتحاليل

Name : Samihah Abdelsamie El-Sayed Ibrahim

Date : 28-02-2026 08:59

Branch : Dokki

Medical No. : 324877

Ref.Physician Prof : Goustav Roussey Egypt

تعاقب مستشفيات : Ins.Company

M.S

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History

- A 78-year-old lady, with history of left breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings

- No abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,

Dr. Alaa Nagi, MD
Nuclear Medicine

PET-CT UNIT



GOLDEN GATE CENTER

∴ Conclusion: -

- Positive study of a known patient with left breast carcinoma showing:
 - Left breast shows metabolically active (FDG avid) soft tissue neoplastic lesions/masses pathologically proved carcinoma.
 - Metabolically active (FDG avid) left axillary nodal metastatic deposits as described .
 - Metabolically inactive (non FDG avid) hepatic focal lesion as described . For close follow up.
 - No other metabolically active (FDG avid) metastatic deposits could be detected all over the body.

Much Obligated;

PET-CT Unit

*Dr. Khaled Yousef.
Radiologist.*

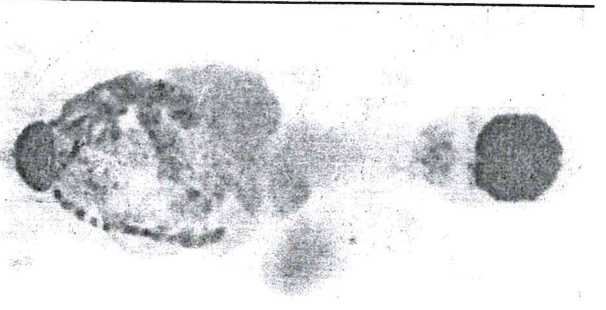
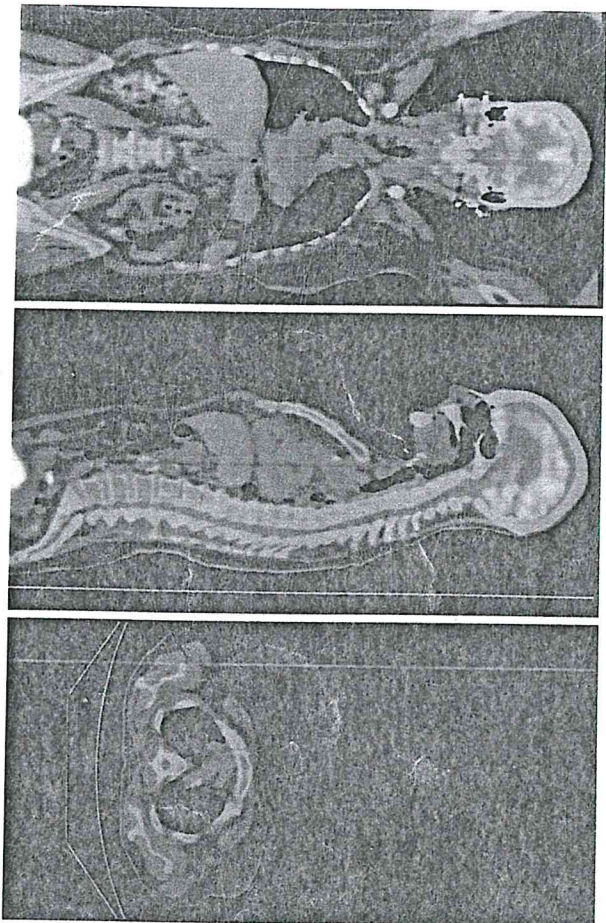
*Dr. Hany Maurice .
Nuclear Medicine Consultant
University of London.*

(+2)0155 1515 732 - (+2)0100 7844 701 - (+2)0122 8290 029 - (+2)24140045 - (+2)24140046
المستشفى المركزي - شارع عمر ابن الخطاب - ارض الجولف - بجوار سيتي ستارز - مدينة نصر



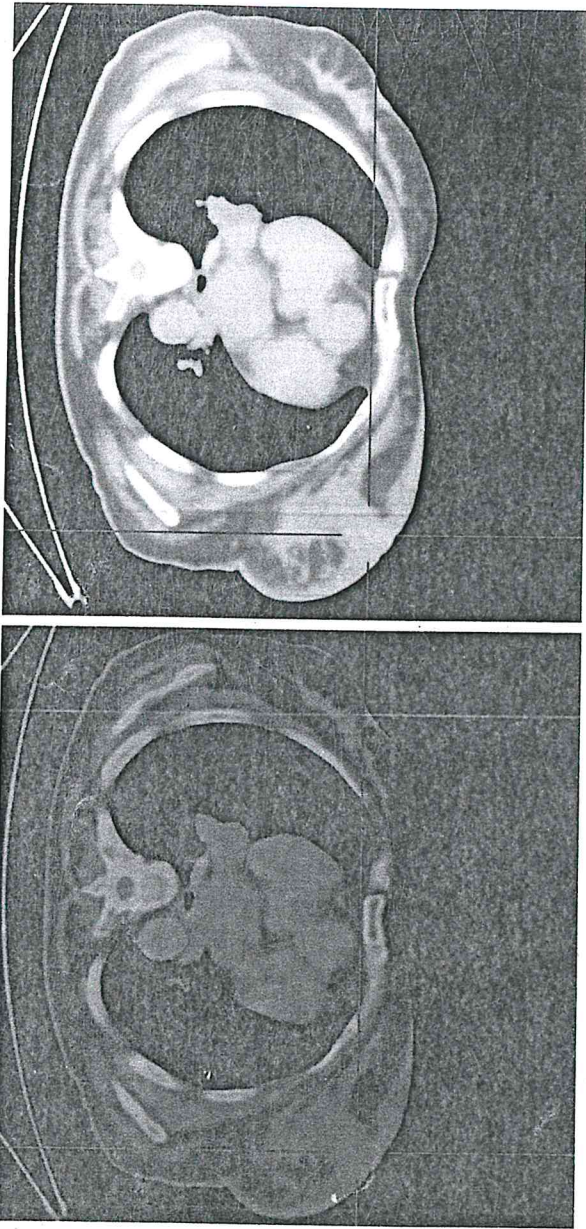


- Normal appearance of the pancreas, aorta and IVC.
 - No metabolically active (FDG avid) ascites.
 - Normal appearance of the pelvic organs with no metabolically active(FDG avid) solid or cystic lesions.
- VI. Scanned axial and Appendicular skeleton:-
- No metabolically active (FDG avid) lytic or sclerotic osseous lesions.
 - Spondylodegenerative changes of several spines.





Findings:-

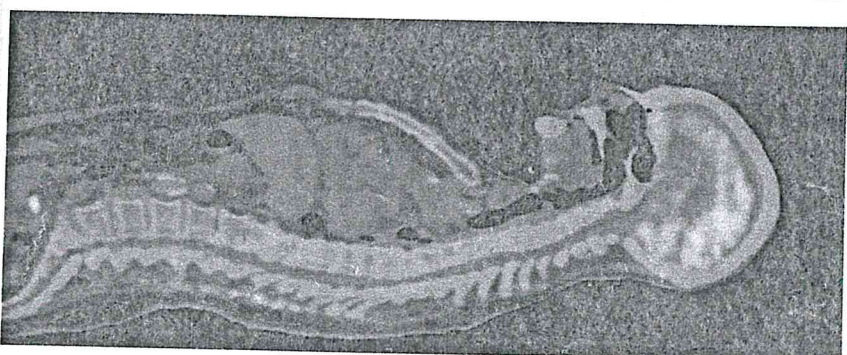
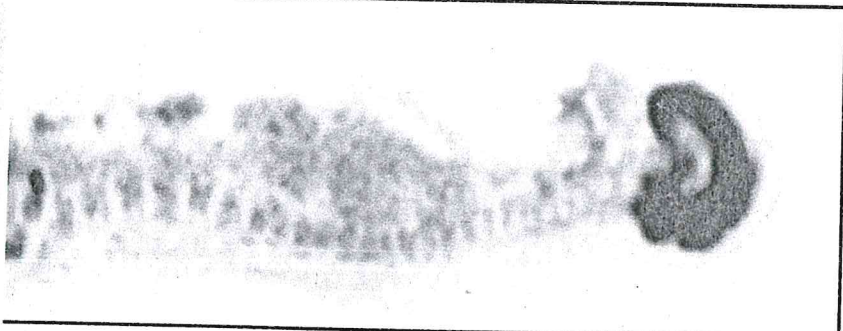
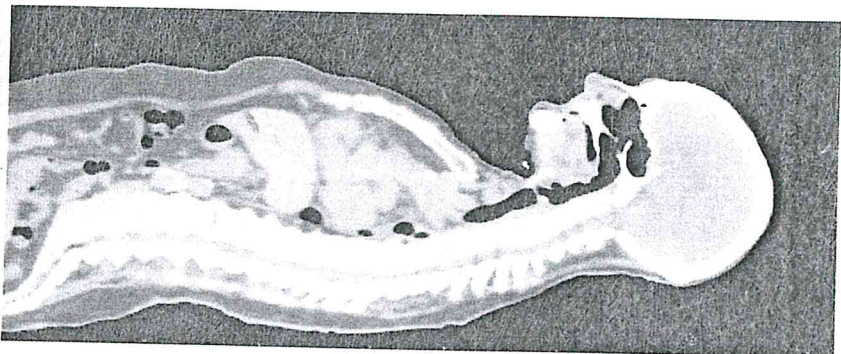
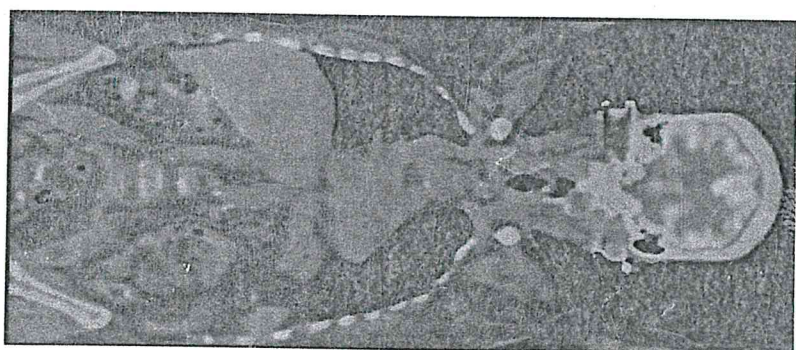
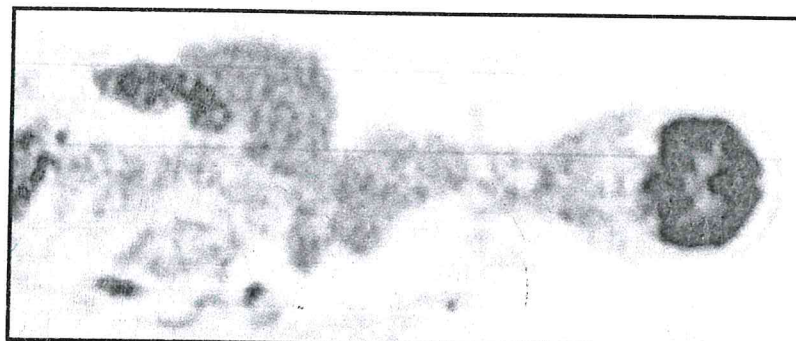
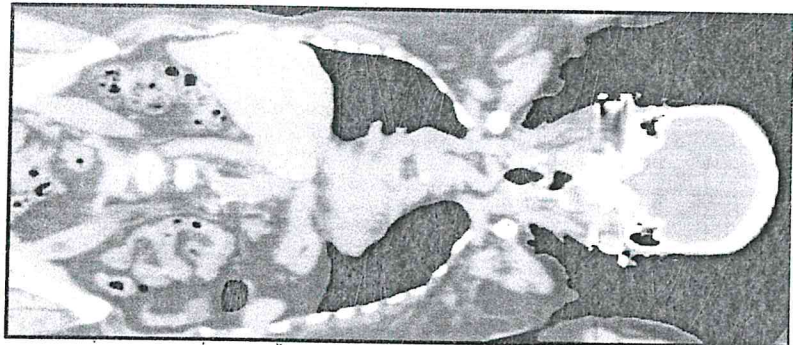


I. Both breasts /1st site of concern:

- Left breast shows:
 - Metabolically active (FDG avid) retro-areolar soft tissue neoplastic mass lesion measuring about 4.8 x 3.2 cm , with max SUV~5.3
 - . Associated with strandy surrounding fat planes with thickened overlying skin . Pathologically proved carcinoma .
 - Metabolically active (FDG avid) left axillary lymph nodes ranging up to 1.4 cm with maximum SUV ~4.5
 - Normal appearance of the right breast with no metabolically active (FDG avid) lesions.
- ### II. Head:
- Uniform tracer bio-distribution over the scanned sections of the cerebral and cerebellar hemispheres.
 - No metabolically active (FDG avid) masses are noted.



BAHRAIN CANCER CENTRE



المستشفى المركزي - شارع عمر ابن الخطاب - ارض الجولف - بجوار سيتي ستارز - مدينة لصر
(+2)0155 1515 732 - (+2)0100 7844 701 - (+2)0122 8290 029 - (+2)24140045 - (+2)24140046



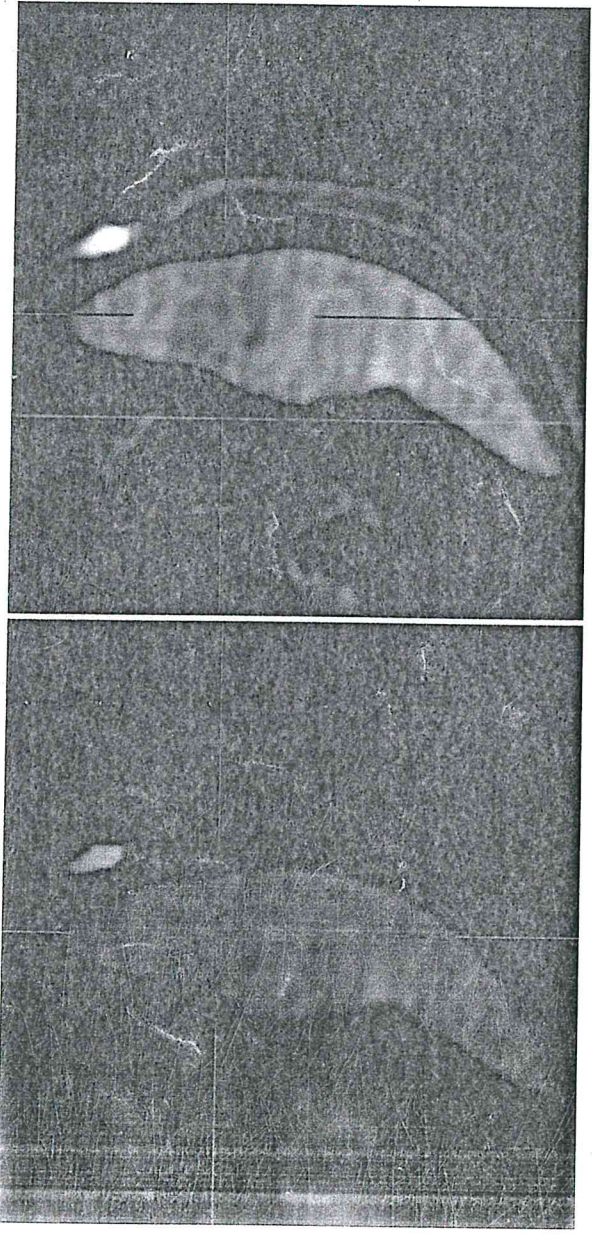


III. Neck:-

- No metabolically active (FDG avid) cervical lymph nodes.
- Normal physiological tracer distribution over the scanned pharyngeal lymphoid tissue as well as salivary glands.

IV. Chest:-

- No metabolically active (FDG avid) pulmonary nodules or masses.
- No metabolically active (FDG avid) mediastinal or hilar lymph nodes.
- No metabolically active (FDG avid) pleural or pericardial effusion.
- Normal physiological tracer uptake of the heart.



V. Abdomen and pelvis:-

- Average size liver showing metabolically inactive (non-FDG avid) focal hepatic lesion at segment VI , measuring around 3.2 x 2.1 cm with no corresponding significant FDG uptake (? Remitted ? benign). No IHBRS.
- Average size spleen showing no metabolically active (FDG avid) focal lesions.
- No metabolically active(FDG avid) abdominal lymph nodes are seen.
- Normal tracer concentration and excretion at both kidneys, ureters and urinary bladder.



MDT FORM

7/1/2026 G

Name	م. السيد عبد الحليم السيد	MRN	٤٤٧٤/٢٤
Age	78y.	ID	٤٤٨.٤.٥٠١٧.١.٤٤٨
Menopausal status	post	Phone	٥١ 222 369411
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity	DH, HTW	REASON OF MDT	

Clinical Data

BX:	DOCT	R	ps:2	L
ER:	PR:	HER 2:	Ki67:	
8/8	8/8	-	3/8	
Sonomammography:				
post left Breast:				
CT4, N3 Mo				
Mets Workup:				
Rec. left AI 1/2025				TNM
L 7/2025 - DR				MDT for 12 w,
Start Taxol weekly x 15 wks.				

Recommendations:	
Surgical Review	will Regressive Coma
Still Residual	OLG: No palpable breast
Lt Breast skin	or axLN.
Edema + central	for MDT for surg.
mass	
For possible	
MDT OUTCOME & PLAN	
Consider for	for CT CAP +
20/12/2025	if proved no mets
note: HFL was Reviewed	Consider for MDT
at the initial MDT → for followup (21)	(No evidence of mets
	from the stalk).

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
	Omnia Essam				
7/1/2026	7/1/2026				



Dr. Islam Radiology Center

مركز الدكتور إسلام الشاذلي للأشعة

دكتوراه الأشعة التشخيصية
إستشاري ومدرس الأشعة
التشخيصية بكلية الطب

Name: الاسم: سميحة عبد السميع

Date: 11-11-2024

Ref: أ.د: عبد الجواد فؤاد

Report

* BILATERAL SOFT TISSUE SONO-MAMMOGRAPHY

- A spiculated ill-defined , soft tissue dense opacity is noted at the upper half of the left breast with distorted breast parenchyma . No detectable micro-calcifications .
- Normal appearance of the right breast parenchyma with predominantly fatty breast
- Enlarged left axillary nodes .

* COMPLEMENTARY ULTRASOUND:

- There is an ill-defined spiculated heterogeneous hypoechoic mass lesion at the left breast upper half having (12 O'clock) periareolar location. Measuring 1.6 x 1.4 cm in maximum dimensions .
- A well defined , cystic ,unilocular breast lesion with turbid fluid content is seen at upper outer quadrant of the left breast at (1 O'clock). It measures (0.7 x 0.5 cm) in maximum dimensions .No detected mural wall thickening or calcification suggesting galactoceles.
- Multiple left axillary, hypoechoic Metastatic lymphadenopathy are also seen (the largest measuring 1.6 x 1.3 cm in maximum dimensions)

* CONCLUSION

- LEFT BREAST MASS LESION (BI-RADS - 5)
- LEFT BREAST GALACTOCELE .

Bi-Rads Classification For Breast Disorders

- | | | |
|---|---|--|
| 0 | - | Need Additional Imaging Evaluation
or Prior Mammograms For Comparison |
| 1 | = | Negative
There is nothing to comment on |
| 2 | = | Benign Finding |
| 3 | = | Probably Benign Finding (<2% malignant)
Initial Short-Interval Follow-Up Suggested |
| 4 | = | Suspicious Abnormality (2 - 95% malignant)
Biopsy Should Be Considered |
| 5 | = | Highly Suggestive of Malignancy (>95% malignant)
Appropriate Action Should Be Taken |
| 6 | = | Known Biopsy - Proven Malignancy |

Thanks

Dr. ISLAM EL SHAZLY (M.D)

*Maadi Military Medical Compound
Oncology and Hematology Hospital
Nuclear Medicine Department*

Patient Name: Samiha Abdel-Samea El-Sayed
Date of Scan: 18/12/2024
Patient ID: 1_429575

^{99m}Tc -MDP SKELETAL SCINTIGRAPHY

History

- 77-year-old lady, with history of left breast cancer, referred for metastatic workup.

Technique

- Anterior and posterior whole body scan were acquired 2 hours post injection of the tracer.

Findings:

- Mild increased tracer uptake is seen at DV19 and DV12; likely degenerative in nature, for correlation with radiological data.
- Otherwise, no abnormal active or cold bony lesions detected over the surveyed body to account for metastatic bone disease.

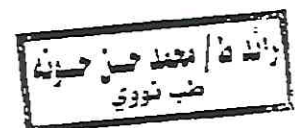
Impression:

- No scintigraphic evidence of metastatic bone disease.

Best regards,

Dr. Ahmed Abdel-Karim

Dr. Mohamed Hassan, MSc





Patient Name	: الأستاذ / سميحة عبدالمصنع السيد ابراهيم	Request No.	: 13008466
Patient No.	: 248040501701243	Request Date	: 20/11/2024
Path No.	: 4499/24	Verification Date	: 07/12/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 76 Y	Department	: Out Patient
Referreing Doc. :		IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 34% nuclear reaction of tumor cells.

Conclusion:

Left breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67: Positive in 34% of tumor cells.

Comment:

- * Scoring system: "ER, PR"

Verified by :

Dr.Heba Hasan

Printing Date : 07/12/2024 10:37:48

Page 1 of 2

Printed By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مهنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

ame: Mrs. samiha abdelameie elsayed

Date: 20-12-2025

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: pathologically proven left breast carcinoma , received chemotherapy and hormonal treatment

Findings:

Right breast.

- Scattered fibroglandular density (ACR b)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated homogenous fatty parenchyma
- no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- No significant axillary lymph nodes

Left breast.

- Scattered fibroglandular density (ACR b)
- **An irregular speculated dense mass is seen in the upper central region with detected internal microcalcifications within**
- **Thickened skin and course trabeculae**
- **Ultrasound demonstrated an irregular spiculated hypoechoic mass at 12 o'clock zone a measuring 17 x 18 mm with detected internal vascularity within**
- **Non specific axillary lymph nodes**

Conclusion:-

Right breast (BIRADS 1): normal

Left breast (BIRADS 6): pathologically proven breast carcinoma

Dr farah elshobaky



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الست المصرية
صحة
مصر

MDT FORM 29/12/2024 G AM

Name	سيدة عبدالمعالي	MRN	5572/C.C.2
Age	77	ID	5572-C.C.2-152
Menopausal status	Post	Phone	011111-111
NRS SCORE		Marital status	Widow
BMI	average	Family history	-ve
CO morbidity	DM - HTN	REASON OF MDT	New case

Clinical Data

BX: L IDC grade grade II with free DCIS				
ER:	PR:	HER 2:	Ki67:	
ve(8/8)	ve(8/8)	-ve	34+	
Sonomammography: Lt Breast speculated ill defined soft tissue dense opacity is noted at upper half of the Lt Breast				TNM
Mets Workup: with distorted breast Parenchyma - Enlarged Lt axillary nodes				
CT CAP :- chest :- Free - Liver :- hepatic focal Lesion for MRI assessment - Bone Scan :- Free				
Recommendations: MRI, - well defined focal lesion 32X72X18cm. Metastatic Breast to liver?!				

MDT OUTCOME & PLAN

AI for 3 Months then re-assess

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

SMC-DSH-F-MDT-001

Revision no:0

Issue no :1

Issue date: 1 / 7 / 2024

MDT Form
[[DOCTYPE:MDT_FORM]]

GAZA FAMILY MILITARY HOSPITAL		وزارة الدفاع إدارة الخدمات الطبية مستشفى الجلاء لعائلات ضباط ق.م.
--	---	--

PATIENT'S NAME: Samiha Abd Almaseeh

DATE: 26 December 2024

1/1

**CONTRAST MRI OF THE ABDOMEN AND PELVIS REVEALED: -
FINDINGS:**

- History of cancer breast.
- The liver is of average size, shows segment VI well defined focal lesion with lobulated outline and delayed heterogenous enhancement, measuring about 32 x22x18 mm at its maximum CC, AP, TR dimensions.
- No intra or extra-hepatic biliary dilatation.
- Average distension and wall thickness gall bladder with large stone.
- Spleen is of normal size and MRI appearance, no focal lesion.
- Pancreas is of normal MRI appearance.
- Normal MRI appearance of both kidneys, showing preserved parenchyma with no focal lesions, stones or back pressure changes.
- Both adrenal glands present normal size and position with normally developed crura. There is no evidence of a mass and no circumscribed expansion.
- Normal flow signal of the major abdominal vessels.
- No MR signs of pathologically enlarged abdominal c lymph nodes.
- No ascites.
- Normal MRI of the visualized bowel loops.

OPINION: -

- Focal hepatic lesion as described, likely deposit for PET /CT correlation.

Dr. Fadila Mamdouh MD



Patient Name	: الأستاذ / سميحة عبدالسميع السيد ابراهيم	Request No.	: 13008466
Patient No.	: 248040501701243	Request Date	: 20/11/2024
Path No.	: 4499/24	Verification Date	: 07/12/2024
Sex	: Female	Financial Category	: اورام دار السلام - ميانره
Age	: 76 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
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- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 34% nuclear reaction of tumor cells.

Conclusion:

Left breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67: Positive in 34% of tumor cells.

Comment:

- * Scoring system: "ER, PR"

Verified by :

Dr.Heba Hasan

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Page 1 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name	: الأستاذة / سميرة عبدالسميع السيد ابراهيم	Request No.	: 13008466
Patient No.	: 248040501701243	Request Date	: 20/11/2024
Path No.	: 4499/24	Verification Date	: 07/12/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبانره
Age	: 76 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast 12 O'clock mass.

Nature of Specimen: Tru-cut biopsy

Gross Examination:

- Four grayish white tissue cores up to 2 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Focal.
- * Perineural invasion: Not detected.
- * Angioinvasion: Not detected.
- * Microcalcifications: Not detected.

Diagnosis:

Left breast 12 O'clock mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II with focal DCIS.

- For further IHC (ER, PR, Her2 and Ki67).

Signature
Prof. Dr. Lobna Omar
Dr. Menna Nabil

Verified by :

Dr. Heba Hasan

Printing Date : 07/12/2024 10:37:53

Page 1 of 2

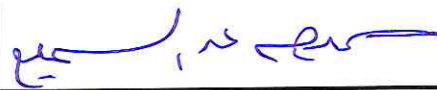
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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

MDT FORM

8 / 11 / 2025 G

Name		MRN	29 44 / 252
Age	48	ID	01118100188
Menopausal status	Post	Phone	01118100177
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity	DM, HTN.	REASON OF MDT	post NACT

Clinical Data

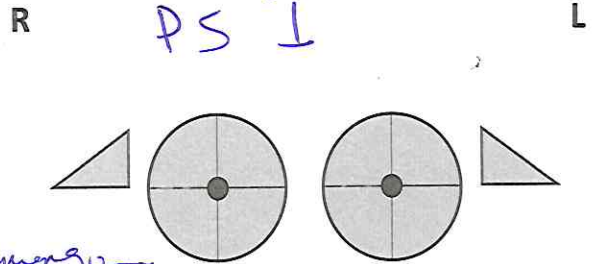
BX: IDC II

ER: +ve. 8/8 PR: 8/8 HER 2: -ve Ki67: 34%

Sonomammography: 8-11-2025 Post scan → HFL
alt Dr Ali Cusul → Mostly benign

Mets Workup:

PT finish NACT
Taxol x 12 wks
last 8-11-2025



TNM
CT4 N3 Mo

Recommendations:

SM → R
R. Surgical Consult

8-11-2025

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT FORM

/ / 20

G

Name	<i>[Signature]</i>	MRN	ΣCUE/CCE
Age	77	ID	
Menopausal status	Post	Phone	-11111-1111
NRS SCORE		Marital status	widow
BMI		Family history	-ve
CO morbidity	Dm - HTN	REASON OF MDT	Post Neo Adj.

Clinical Data

BX: IDC GII e fcr DCIS			
ER: 8/8	PR: 8/8	HER 2: -ve	Ki67: 39%
Sonomammography: lt breast mass / lt axillary lvs			
Mets Workup: new / PET CT			
Pt on AI - lt breast mass for bms - lt axillary node			
Recommendations: - Inactive hepatic foci (now) pt advance inflammatory changes on lt breast ultrasound for your recommendation.			
MDT OUTCOME & PLAN Intich CT → HFLs MRI was done consider it Mets start AI PET-CT → non avoid HFLs so ① us guided biops ② New S.M ③ bring files / MRI دكتور محمد عبد الله			

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



Patient Name: Samiha Abd Al Sameea Al Sayed	Date : 08/12/2024
Exam : C.T CHEST WITH CONTRAST	Age : 76 Year(s)
Medical ID: 4274/2024	Pat. ID: 95181

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- left breast heterogenous parenchyma with mild increase in skin thickness with ipsilateral enlarged level I, level II and level III axillary lymph nodes largest on at level I measures about 1.8x 1.5 cm.
- prominent left supraclavicular lymph nodes , the largest measures about 1.5x 1.2 cm.
- Prominent right axillary lymph nodes.
- Few scattered millimetric tiny pulmonary nodules noted , no masses, areas of consolidation or cavitory lesions.
- Prominent prevascular and subcarinal lymph nodes
- Bilateral basal ground glass opacities noted.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Mild cardiomegaly with no gross abnormality of the cardiac chambers. No pericardial effusion detected.

II Abdomen & pelvis:

- The liver is of average size with partly exophytic hypodense focal lesion seen at segment VI measures about 2.9x 2.3 cm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** well defined lytic lesion noted at the body of L1, and bilateral acetabulum

Patient's name **سميحة عبد السميع السيد ابراهيم**

Referred by : **أد/ خالد جميل**

Date : **21 November-2024**

رقم الملف: **4400**

Abdominal and pelvic ultrasound revealed:

- **The liver** is mild enlarged in size, *bright* echo pattern, regular surface, no peri-portal fibrosis, no focal lesions, biliary dilatation or hepatic venous congestion could be seen. IVC is not congested.
- **The PV** is patent and not dilated, of 10 mm.
- **The gall bladder** is average in size with average wall thickness, with stone measures 49.2 mm with multiple tiny stones of average 2.8 mm could be seen. CBD is not dilated.
- **The spleen** is of average size with its span of 93 mm with homogenous echopattern and no focal lesion could be seen.
- **The splenic vein** is not dilated.
- **The pancreas & Para aortic area** are u-s free.
- **The kidneys** are of normal size, well differentiated & with no backpressure or space occupying lesion could be seen. **Multiple gravels** are seen with renal sinus splitting; picture consistent with repeated low grade urinary tract infection.
- **No Ascites** detected.
- **The urinary bladder** show average filling capacity, regular average sized wall thickness, no stones, polyps, diverticulae or masses could be seen.
- **The uterus** is AVF of average size, measuring 60x30 mm, of homogenous echo pattern: no focal lesion seen with clear average sized endometrial line.
- **Both ovaries** are of average size with no focal lesion or cysts could be seen
- **Douglass pouch** is free
- **No adnexal masses**

Impression:

- **Bright enlarged liver.**
- **Calcular gall bladder.**
- **Picture consistent with repeated low grade urinary tract infection.**
- **Marked colonic distension.**

Much obliged.

Dr/ Mohammad Kinawy



Dr. Islam Radiology Center

مركز الدكتور إسلام الشاذلي للأشعة

دكتوراه الأشعة التشخيصية
إستشاري ومدرس الأشعة
التشخيصية بكلية الطب

Name: الاسم: سميحة عبد السميع
Date: 11-11-2024
Ref: أ.د. عبد الجواد فؤاد

Report

* BILATERAL SOFT TISSUE SONO-MAMMOGRAPHY *

- A spiculated ill-defined , soft tissue dense opacity is noted at the upper half of the left breast with distorted breast parenchyma . No detectable micro-calcifications .
- Normal appearance of the right breast parenchyma with predominantly fatty breast
- Enlarged left axillary nodes .

* COMPLEMENTARY ULTRASOUND:

- There is an ill-defined spiculated heterogeneous hypoechoic mass lesion at the left breast upper half having (12 O'clock) periareolar location. Measuring 1.6 x 1.4 cm in maximum dimensions .
- A well defined , cystic ,unilocular breast lesion with turbid fluid content is seen at upper outer quadrant of the left breast at (1 O'clock) . It measures (0.7 x 0.5 cm) in maximum dimensions .No detected mural wall thickening or calcification suggesting galactoceale.
- Multiple left axillary, hypoechoic Metastatic lymphadenopathy are also seen (the largest measuring 1.6 x 1.3 cm in maximum dimensions)

* CONCLUSION

- LEFT BREAST MASS LESION (BI-RADS - 5)
- LEFT BREAST GALACTOCELE .

Bi-Rads Classification For Breast Disorders

- | | |
|-----|--|
| 0 = | Need Additional Imaging Evaluation
or Prior Mammograms For Comparison |
| 1 = | Negative
There is nothing to comment on |
| 2 = | Benign Finding |
| 3 = | Probably Benign Finding (<2% malignant)
Initial Short-Interval Follow-Up Suggested |
| 4 = | Suspicious Abnormality (2 - 95% malignant)
Biopsy Should Be Considered |
| 5 = | Highly Suggestive of Malignancy (>95% malignant)
Appropriate Action Should Be Taken |
| 6 = | Known Biopsy - Proven Malignancy |

Thanks
Dr. ISLAM EL SHAZLY (M.D.)



٢٠٢٤/٠٣ ٢٤٨.٤.٥١٧.١٢٤٣

ربه منزل

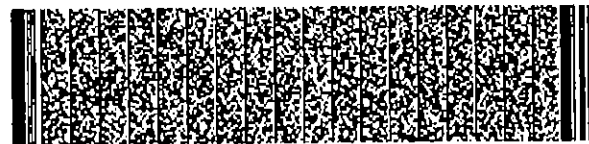
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مسلمة

أرملة

البطاقة سارية حتى ٢٠٢٧/٠٣/١٠

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ERM036728336



جمهورية مصر العربية
بطاقة تحقيق الشخصية

اسمها

عبدالسميع السيد البراهيم

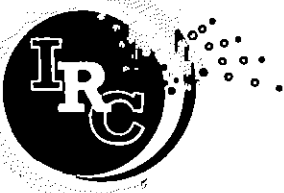
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شبرا الخيمة اول - القليوبية



KM4723189

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Dr. Islam Radiology Center

مركز الدكتور إسلام الشاذلي للأشعة

دكتوراه الأشعة التشخيصية
إستشاري ومدرس الأشعة
التشخيصية بكلية الطب

Name: الاسم: سميحه عبدالسميع

Date: 11-11-2024

Ref: أ.د. عبد الجواد فؤاد

Report

BILATERAL SOFT TISSUE SONO-MAMMOGRAPHY

- A spiculated ill-defined , soft tissue dense opacity is noted at the upper half of the left breast with distorted breast parenchyma . No detectable micro-calcifications .
- Normal appearance of the right breast parenchyma with predominantly fatty breast
- Enlarged left axillary nodes .

* COMPLEMENTARY ULTRASOUND:

- There is an ill-defined spiculated heterogeneous hypoechoic mass lesion at the left breast upper half having (12 O'clock) periareolar location. Measuring 1.6 x 1.4 cm in maximum dimensions .
- A well defined , cystic ,unilocular breast lesion with turbid fluid content is seen at upper outer quadrant of the left breast at (1 O'clock). It measures (0.7 x 0.5 cm) in maximum dimensions .No detected mural wall thickening or calcification suggesting galactoceles.
- Multiple left axillary,hypoechoic Metastatic lymphadenopathy are also seen(the largest measuring 1.6 x 1.3 cm in maximum dimensions)

* CONCLUSION

- LEFT BREAST MASS LESION (BI-RADS - 5)
- LEFT BREAST GALACTOCELE .

Bi-Rads Classification For Breast Disorders

- | | | |
|---|---|---|
| 0 | - | Need Additional Imaging Evaluation
or Prior Mammograms For Comparison |
| 1 | = | Negative
There is nothing to comment on |
| 2 | = | Benign Finding |
| 3 | = | Probably Benign Finding (<2% malignant)
Initial Short-Interval Follow-Up Suggested |
| 4 | = | Suspicious Abnormality (2 - 95% malignant)
Biopsy Should Be Considered |
| 5 | = | Highly Suggestive of Malignancy(>95% malignant)
Appropriate Action Should Be Taken |
| 6 | = | Known Biopsy - Proven Malignancy |

Thanks

Dr. ISLAM EL SHAZLY (M D)

Patient's name **سميحة عبد السميع السيد ابراهيم**

Referred by **أد/ خالد جميل**

Date **: 21 November 2024**

رقم الملف: **4400**

Abdominal and pelvic ultrasound revealed:

- **The liver** is mild enlarged in size, *bright* echo pattern, regular surface, no peri-portal fibrosis, no focal lesions, biliary dilatation or hepatic venous congestion could be seen. IVC is not congested.
- **The PV** is patent and not dilated, of 10 mm.
- **The gall bladder** is average in size with average wall thickness, with **stone measures 49.2 mm with multiple tiny stones of average 2.8 mm** could be seen. CBD is not dilated.
- **The spleen** is of average size with its span of 93 mm with homogenous echopattern and no focal lesion could be seen.
- **The splenic vein** is not dilated.
- **The pancreas & Para aortic area** are u-s free.
- **The kidneys** are of normal size, well differentiated & with no backpressure or space occupying lesion could be seen. **Multiple gravels are seen with renal sinus splitting; picture consistent with repeated low grade urinary tract infection.**
- **No Ascites** detected.
- **The urinary bladder** show average filling capacity, regular average sized wall thickness, no stones, polyps, diverticulae or masses could be seen.
- **The uterus** is AVF of average size, measuring 60x30 mm, of homogenous echo pattern: no focal lesion seen with clear average sized endometrial line.
- **Both ovaries** are of average size with no focal lesion or cysts could be seen
- **Douglass pouch** is free
- **No adnexal masses**

Impression:

- **Bright enlarged liver.**
- **Calcular gall bladder.**
- **Picture consistent with repeated low grade urinary tract infection.**
- **Marked colonic distension.**

Much obliged.

Dr/ Mohammad Kinawy



Patient Name: Samiha Abd Al Sameea Al Sayed	Date : 08/12/2024
Exam : C.T CHEST WITH CONTRAST	Age : 76 Year(s)
Medical ID: 4274/2024	Pat. ID: 95181

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- left breast heterogenous parenchyma with mild increase in skin thickness with ipsilateral enlarged level I, level II and level III axillary lymph nodes largest on at level I measures about 1.8x 1.5 cm.
- prominent left supraclavicular lymph nodes , the largest measures about 1.5x 1.2 cm.
- Prominent right axillary lymph nodes.
- Few scattered millimetric tiny pulmonary nodules noted , no masses, areas of consolidation or cavitory lesions.
- Prominent prevascular and subcarinal lymph nodes
- Bilateral basal ground glass opacities noted.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Mild cardiomegaly with no gross abnormality of the cardiac chambers. No pericardial effusion detected.

II Abdomen & pelvis:

- The liver is of average size with partly exophytic hypodense focal lesion seen at segment VI measures about 2.9x 2.3 cm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** well defined lytic lesion noted at the body of L1,
and bilateral acetabulum



Patient Name: Samiha Abd Al Sameea Al Sayed	Date : 08/12/2024
Exam : C.T CHEST WITH CONTRAST	Age : 76 Year(s)
Medical ID: 4274/2024	Pat. ID: 95181

Opinion:

- Left breast malignant process with ipsilateral axillary and supraclavicular lymph nodes.
- Few bilateral tiny pulmonary nodules for clinical correlation
- Hepatic focal lesion as described worrisome of being metastatic for further triphasic MRI assessment.
- Few osseous lytic lesions for further assessment with bone scan.

Dr. Marwa Kamal

Prof. Menna Talla Hanafy, M.D.

GALA FAMILY MILITARY HOSPITAL		وزارة الدفاع إدارة الخدمات الطبية مستشفى الجلاء لعائلات ضباط ق.م
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PATIENT'S NAME: Samiha Abd Almaseeh

DATE: 26 December 2024

أعزى

CONTRAST MRI OF THE ABDOMEN AND PELVIS REVEALED: -
FINDINGS:

- History of cancer breast.
- The liver is of average size, shows segment VI well defined focal lesion with lobulated outline and delayed heterogenous enhancement, measuring about 32 x22x18 mm at its maximum CC, AP, TR dimensions.
- No intra or extra-hepatic biliary dilatation.
- Average distension and wall thickness gall bladder with large stone.
- Spleen is of normal size and MRI appearance, no focal lesion.
- Pancreas is of normal MRI appearance.
- Normal MRI appearance of both kidneys, showing preserved parenchyma with no focal lesions, stones or back pressure changes.
- Both adrenal glands present normal size and position with normally developed crura. There is no evidence of a mass and no circumscribed expansion.
- Normal flow signal of the major abdominal vessels.
- No MR signs of pathologically enlarged abdominal c lymph nodes.
- No ascites.
- Normal MRI of the visualized bowel loops.

OPINION: -

- Focal hepatic lesion as described, likely deposit for PET /CT correlation.

Dr. Fadila Mamdouh MD

MDT FORM

/ / 20

G

Name		MRN	ΣCUE/CCE
Age	77	ID	
Menopausal status	Post	Phone	111 111 - 1111
NRS SCORE		Marital status	widow
BMI		Family history	-ve
CO morbidity	Dm - HTN	REASON OF MDT	Post Neo Ad5.

Clinical Data

BX: IDC GII e fck DCIS				L	
ER: 8/8	PR: 8/8	HER 2: -ve	Ki67: 39%		
Sonomammography:					
lt breast mass / lt axillary lms					
Mets Workup: new / PET CT				TNM	
Pt on AI - lt breast mass					
for bms - lt axillary node					
Recommendations:				Inactive hepatic fcn (now) pt advance inflammatory changes on lt breast submit for your recommendation.	
MDT OUTCOME & PLAN				In: ch' ct → HFLs MRI was done consider it Mets start AI PET-CT → non avid HFLs so ① us guided biops ② New S.M ③ bring files / MRI مريضنا سيجي للمتابعة	

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



Patient Name	: الأستاذ / سميحة عبدالمميع السيد ابراهيم	Request No.	: 13008466
Patient No.	: 248040501701243	Request Date	: 20/11/2024
Path No.	: 4499/24	Verification Date	: 07/12/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 76 Y	Department	: Out Patient
Referreing Doc. :		IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.
Nature of Specimen: Tru-cut biopsy.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 34% nuclear reaction of tumor cells.

Conclusion:

Left breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II.
IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67: Positive in 34% of tumor cells.

Comment:

- * Scoring system: "ER, PR"

Verified by :

Dr.Heba Hasan

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Page 1 of 2

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Patient Name	الأستاذة / سميحة عبدالمميع السيد ابراهيم	Request No.	13008466
Patient No.	248040501701243	Request Date	20/11/2024
Path No.	4499/24	Verification Date	07/12/2024
Sex	Female	Financial Category	اورام دار السلام - مبادره
Age	76 Y	Department	Out Patient
Referreing Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within > 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Prof. Dr. Amal Hareedy
Dr. Menna Nabil

Verified by :
Dr.Heba Hasan

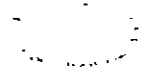
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Page 2 of 2

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name	: الأستاذ / سميه عبدالسميع السيد ابراهيم	Request No.	: 13008466
Patient No.	: 248040501701243	Request Date	: 20/11/2024
Path No.	: 4499/24	Verification Date	: 07/12/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 76 Y	Department	: Out Patient
Referring Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast 12 O'clock mass.

Nature of Specimen: Tru-cut biopsy

Gross Examination:

- Four grayish white tissue cores up to 2 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Focal.
- * Perineural invasion: Not detected.
- * Angioinvasion: Not detected.
- * Microcalcifications: Not detected.

Diagnosis:

Left breast 12 O'clock mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II with focal DCIS.

- For further IHC (ER, PR, Her2 and Ki67).

Signature
Prof. Dr. Lobna Omar
Dr. Menna Nabil

Verified by :

Dr. Heba Hasan



*Maadi Military Medical Compound
Oncology and Hematology Hospital
Nuclear Medicine Department*

Patient Name: Samiha Abdel-Samea El-Sayed

Date of Scan: 18/12/2024

Patient ID: 1_429575

^{99m}Tc-MDP SKELETAL SCINTIGRAPHY

History

- 77-year-old lady, with history of left breast cancer, referred for metastatic workup.

Technique

- Anterior and posterior whole body scan were acquired 2 hours post injection of the tracer.

Findings:

- Mild increased tracer uptake is seen at DV19 and DV12; likely degenerative in nature, for correlation with radiological data.
- Otherwise, no abnormal active or cold bony lesions detected over the surveyed body to account for metastatic bone disease.

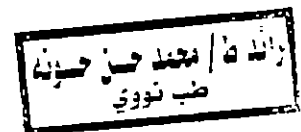
Impression:

- No scintigraphic evidence of metastatic bone disease.

Best regards,

Dr. Ahmed Abdel-Karim

Dr. Mohamed Hassan, MSc





Dr. Islam Radiology Center

مركز الدكتور إسلام الشاذلي للأشعة

دكتوراه الأشعة التشخيصية
إستشاري ومدرس الأشعة
التشخيصية بكلية الطب

Name: الاسم: سميحه عبدالسميع
Date: 11-11-2024
Ref: أ.د. عبدالجواد فؤاد

Report

BILATERAL SOFT TISSUE SONO-MAMMOGRAPHY

- A spiculated ill-defined , soft tissue dense opacity is noted at the upper half of the left breast with distorted breast parenchyma . No detectable micro-calcifications .
- Normal appearance of the right breast parenchyma with predominantly fatty breast
- Enlarged left axillary nodes .

* COMPLEMENTARY ULTRASOUND:

- There is an ill-defined spiculated heterogeneous hypoechoic mass lesion at the left breast upper half having (12 O'clock) periareolar location. Measuring 1.6 x 1.4 cm in maximum dimensions .
- A well defined , cystic ,unilocular breast lesion with turbid fluid content is seen at upper outer quadrant of the left breast at (1 O'clock) . It measures (0.7 x 0.5 cm) in maximum dimensions .No detected mural wall thickening or calcification suggesting galactocele.
- Multiple left axillary, hypoechoic Metastatic lymphadenopathy are also seen (the largest measuring 1.6 x 1.3 cm in maximum dimensions)

* CONCLUSION

- LEFT BREAST MASS LESION (BI-RADS - 5)
- LEFT BREAST GALACTOCELE .

Bi-Rads Classification For Breast Disorders

- | | |
|-----|--|
| 0 = | Need Additional Imaging Evaluation or Prior Mammograms For Comparison |
| 1 = | Negative
There is nothing to comment on |
| 2 = | Benign Finding |
| 3 = | Probably Benign Finding (<2% malignant)
Initial Short-Interval Follow-Up Suggested |
| 4 = | Suspicious Abnormality (2 - 95% malignant)
Biopsy Should Be Considered |
| 5 = | Highly Suggestive of Malignancy (>95% malignant)
Appropriate Action Should Be Taken |
| 6 = | Known Biopsy - Proven Malignancy |

Thanks

Dr. ISLAM/EL SHAZLY (M.D.)



Patient's Name: Mrs/ Samiha Abd El Samea .

Date; 23/6/2025.

Whole Body PET/CT Examination

Clinical history:-

- A 78 years old female, gave history of pathologically proved left breast carcinoma .
Received chemotherapy .

Reason of study:

- Work up.

Technique:-

- Approximately one hour after the IV injection of the radiopharmaceutical, whole body PET-CT was acquired from skull base to mid-thigh with the arms extended above the head.
- CT was acquired for the purpose of anatomic localization, attenuation correction and diagnostic purpose.
- Data was processed and displayed according to departmental protocol.
- **Radiopharmaceutical:** F-18-FDG 5 mCi injected I.V.

Weight:	67 Kg	Injection Site	Left hand
Height:	160 Cm	IV Contrast	No
Bl.Gluc.level	134	Biodistribution Time	60 min



Findings:-



I. Both breasts /1st site of concern:

○ Left breast shows:

- Metabolically active (FDG avid) retro-areolar soft tissue neoplastic mass lesion measuring about 4.8 x 3.2 cm , with max SUV~ 5.3 . Associated with strandy surrounding fat planes with thickened overlying skin . Pathologically proved carcinoma .
- Metabolically active (FDG avid) left axillary lymph nodes ranging up to 1.4 cm with maximum SUV ~ 4.5

- ##### ○ Normal appearance of the right breast with no metabolically active (FDG avid) lesions.

II. Head :

- ##### ○ Uniform tracer bio-distribution over the scanned sections of the cerebral and cerebellar hemispheres.
- ##### ○ No metabolically active (FDG avid) masses are noted.



∴ Conclusion: -

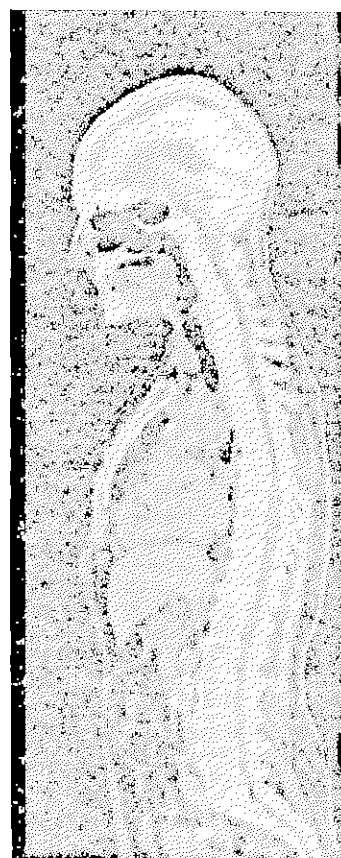
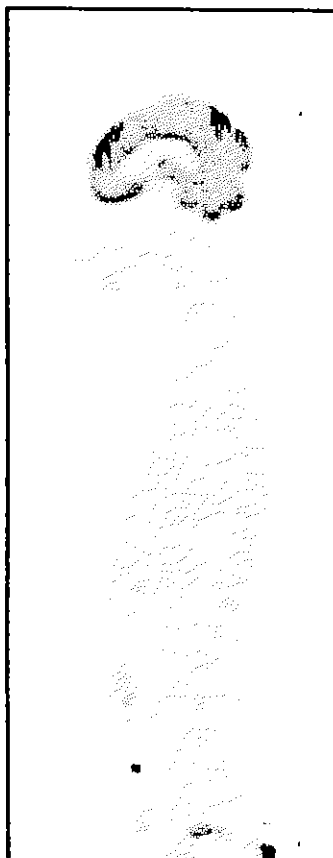
- **Positive study of a known patient with left breast carcinoma showing:**
 - **Left breast shows metabolically active (FDG avid) soft tissue neoplastic lesions/masses pathologically proved carcinoma.**
 - **Metabolically active (FDG avid) left axillary nodal metastatic deposits as described .**
 - **Metabolically inactive (non FDG avid) hepatic focal lesion as described . For close follow up.**
 - **No other metabolically active (FDG avid) metastatic deposits could be detected all over the body.**

Much Obligated;

PET-CT Unit

*Dr. Khaled Yousef.
Radiologist.*

*Dr. Hany Maurice .
Nuclear Medicine Consultant
University of London.*



☎ (+2)24140046 - (+2)24140045 - (+2)0122 8290 029 - (+2)0100 7844 701 - (+2)0155 1515 732

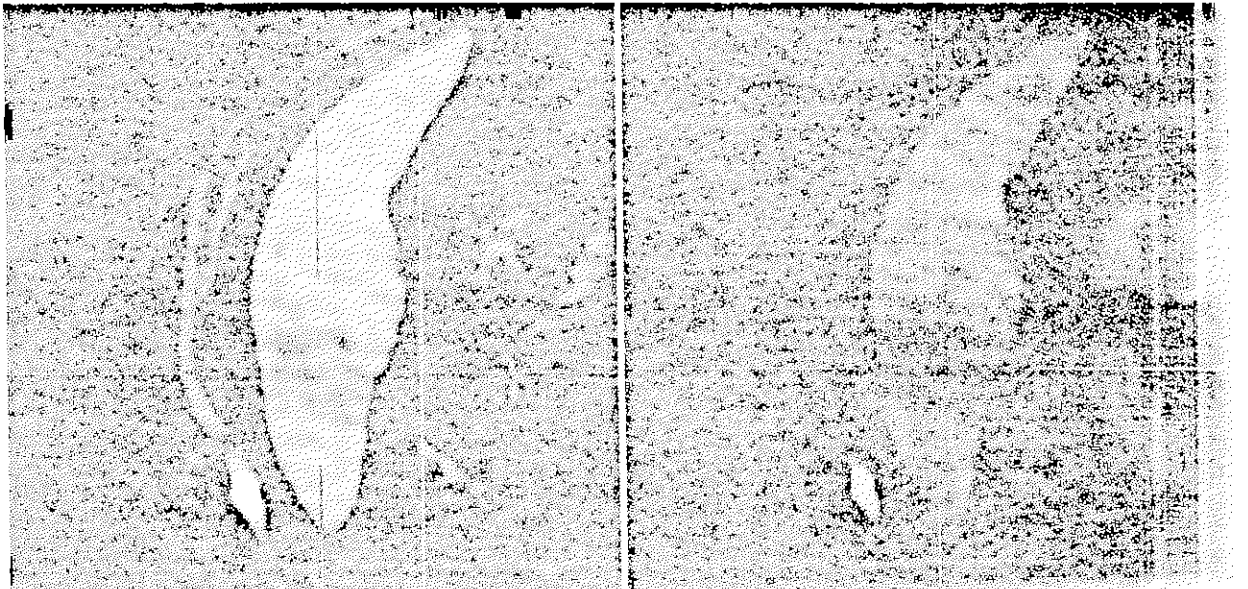
📍 المستشفى المركزي - شارع عمر ابن الخطاب - ارض الجولف - بجوار سيتي ستارز - مدينة نصر

III. Neck:-

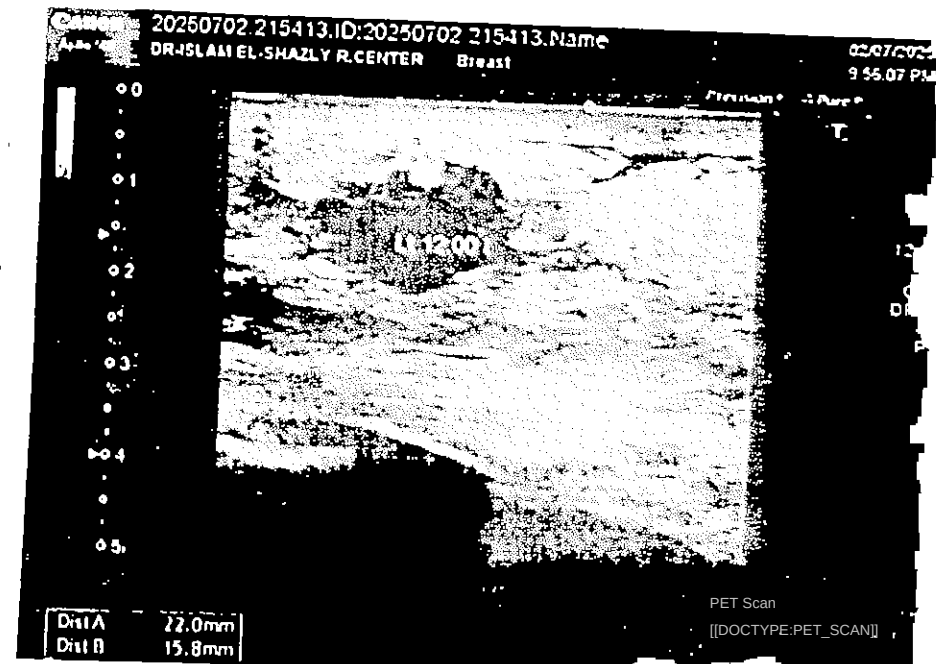
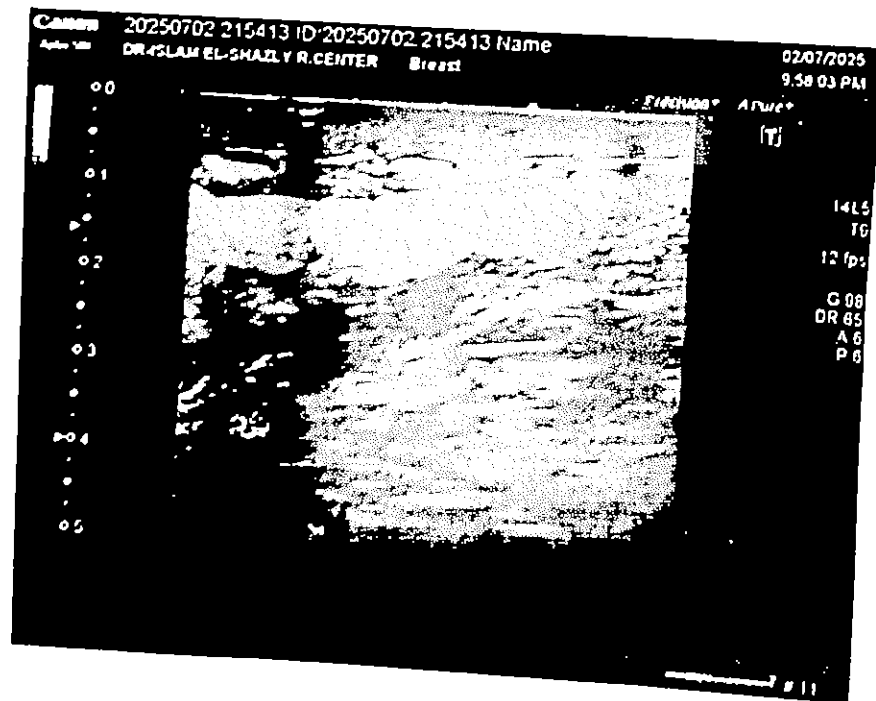
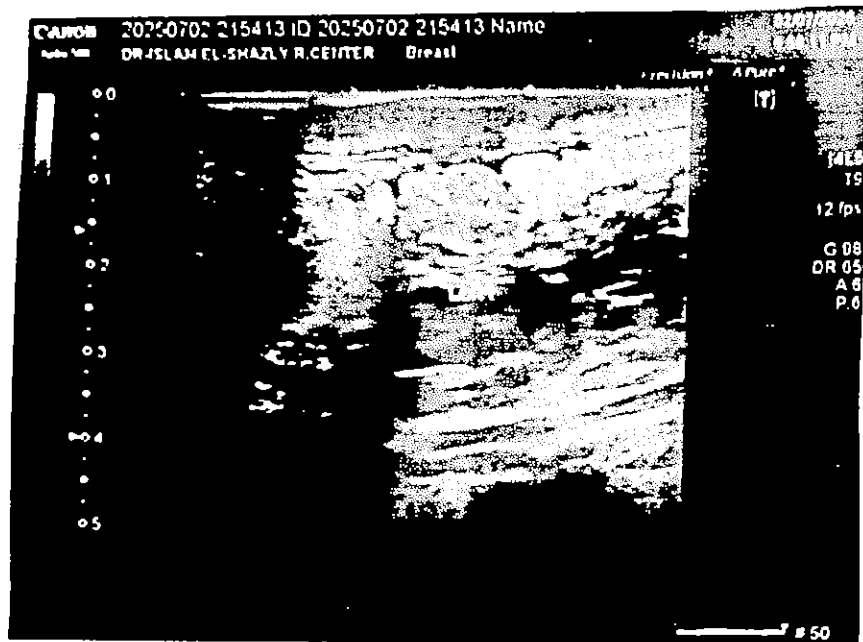
- No metabolically active (FDG avid) cervical lymph nodes.
- Normal physiological tracer distribution over the scanned pharyngeal lymphoid tissue as well as salivary glands.

IV. Chest:-

- No metabolically active (FDG avid) pulmonary nodules or masses.
- No metabolically active (FDG avid) mediastinal or hilar lymph nodes.
- No metabolically active (FDG avid) pleural or pericardial effusion.
- Normal physiological tracer uptake of the heart.

**V. Abdomen and pelvis:-**

- Average size liver showing metabolically inactive (non-FDG avid) focal hepatic lesion at segment VI , measuring around 3.2 x 2.1 cm with no corresponding significant FDG uptake (? Remitted ? benign). No IHBRs.
- Average size spleen showing no metabolically active (FDG avid) focal lesions.
- No metabolically active(FDG avid) abdominal lymph nodes are seen.
- Normal tracer concentration and excretion at both kidneys, ureters and urinary bladder.





Dr. Islam Radiology Center

الاسم: سميحة عبدالسميع

Name: 2-7-2025

Date:

Ref:

مركز الدكتور إسلام الشاذلي للأشعة

دكتوراه الأشعة التشخيصية
إستشاري ومدرس الأشعة
التشخيصية بكلية الطب

* FOLLOW UP LEFT BREAST TISSUE SONO- MAMMOGRAPHY

- A spiculated ill-defined , soft tissue dense opacity is noted at the upper half of the left breast with distorted breast parenchyma . No detectable micro-calcifications .
- Enlarged left axillary nodes .

* COMPLEMENTARY ULTRASOUND:

- There is an ill-defined spiculated heterogeneous hypoechoic mass lesion at the left breast upper half having (12 O'clock) periareolar location. Measuring 2.2 x 1.5 cm in maximum dimensions .
- Marked diffuse area of decreased echogenicity at the left breast with diffuse edema of the overlying skin and subcutaneous tissue together with no fluid collection suggesting mastitis .
- Multiple left axillary, hypoechoic Metastatic lymphadenopathy are also seen (the largest measuring 1.2 x 0.8 cm in maximum dimensions)

* CONCLUSION

- PROGRESSIVE STATUS SINCE THE LAST STUDY .
- LEFT BREAST MASS LESION (BI-RADS - 5)
- MARKED DIFFUSE LEFT BREAST MASTITIS .

Bi-Rads Classification For Breast Disorders

- | | |
|---|--|
| 0 | Need Additional Imaging Evaluation
or Prior Mammograms For Comparison |
| 1 | Negative
There is nothing to comment on |
| 2 | Benign Finding |
| 3 | Probably Benign Finding (<2% malignant)
Initial Short-Interval Follow-Up Suggested |
| 4 | Suspicious Abnormality (2 - 95% malignant)
Biopsy Should Be Considered |
| 5 | Highly Suggestive of Malignancy (>95% malignant)
Appropriate Action Should Be Taken |
| 6 | Known Biopsy - Proven Malignancy |

Thanks

Dr. ISLAM EL SHAZLY (M.D.)



الست المصرية
طحة
مطر

<u>Name</u>	<i>William H. Green</i>	<u>MRN</u>	<i>2005/C.C.2</i>
		<u>ID</u>	<i>SSA-2004-1015</i>
<u>Age</u>	<i>77</i>	<u>Phone</u>	<i>111/11-111</i>
<u>Menopausal status</u>	<i>Post</i>	<u>Marital status</u>	<i>Widow</i>
<u>NRS SCORE</u>		<u>Family history</u>	<i>-ve</i>
<u>BMI</u>	<i>average</i>	<u>REASON OF MDT</u>	<i>New case</i>
<u>CO morbidity</u>	<i>DM - HTN</i>		

BX: Lt IDC grade grade II with focal DCIS				L
ER: +ve (8/8)	PR: +ve (8/8)	HER 2: -ve	Ki67: 34%	
Sonomammography: Lt Breast speiculated ill defined soft tissue dense opacity is noted at upper half of the Lt Breast				
Mets Workup: with distorted breast parenchyma - Enlarged Lt axillary nodes				
CT CAP :- chest :- free - Liver :- hepatic focal lesion for MRI assessment - Bone scan :- free				TNM

Recommendations: MRI, - well defined focal lesion 32x72x18cm.
Metastatic Breast to liver?!

AI for 3 Months then re-assess

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
<i>[Signature]</i>		<i>[Signature]</i>			



الجمهورية العربية السورية
مجلس الوزراء

سميحه

عبدالمصيع السيد ابراهيم

٢٥ ح منسوخ الانتاج - عزبه عثمان
شبرا الخيمة اول - القليوبية



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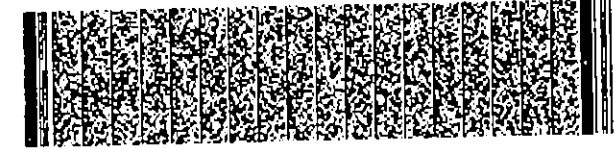
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ربه منزل



أنثى مسلمة أرمنية

البطاقة سارية حتى ٢٠٣١/٠٣/١٠



٧٧
بنية السجاري
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بنية سامية
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Patient Name: Samiha Abd Al Sameea Al Sayed	Date : 08/12/2024
Exam : C.T CHEST WITH CONTRAST	Age : 76 Year(s)
Medical ID: 4274/2024	Pat. ID: 95181

Opinion:

- Left breast malignant process with ipsilateral axillary and supraclavicular lymph nodes.
- Few bilateral tiny pulmonary nodules for clinical correlation
- Hepatic focal lesion as described worrisome of being metastatic for further triphasic MRI assessment.
- Few osseous lytic lesions for further assessment with bone scan.

Dr. Marwa Kamal

Prof. Menna Talla Hanafy, M.D.



Patient Name: Samiha Abd Al Sameea Al Sayed	Date : 08/12/2024
Exam : C.T CHEST WITH CONTRAST	Age : 76 Year(s)
Medical ID: 4274/2024	Pat. ID: 95181

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- left breast heterogenous parenchyma with mild increase in skin thickness with ipsilateral enlarged level I, level II and level III axillary lymph nodes largest on at level I measures about 1.8x 1.5 cm.
- prominent left supraclavicular lymph nodes, the largest measures about 1.5x 1.2 cm.
- Prominent right axillary lymph nodes.
- Few scattered millimetric tiny pulmonary nodules noted, no masses, areas of consolidation or cavitory lesions.
- Prominent prevascular and subcarinal lymph nodes
- Bilateral basal ground glass opacities noted.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Mild cardiomegaly with no gross abnormality of the cardiac chambers. No pericardial effusion detected.

II Abdomen & pelvis:

- The liver is of average size with partly exophytic hypodense focal lesion seen at segment VI measures about 2.9x 2.3 cm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** well defined lytic lesion noted at the body of L1, and bilateral acetabulum

MDT FORM

22/ 2/20 G

Name	اسم المريض	MRN	٤٤٧٤/٢٠٢٤
Age	٧٨	ID	٥١١١٨/٥٥١٨٨
Menopausal status	post	Phone	٥١١١٨١٥٥١٧٧
NRS SCORE		Marital status	٢٤٨٠٤٠٥١٧٠١٤٨
BMI		Family history	
CO morbidity		REASON OF MDT	Local pD

Clinical Data

BX:	ID CII	R	PS: 2	L
ER: tue 8/1	PR: 8/1	HER 2: -	Ki67: 34%	
Sonomammography:				
Mets Workup:		CT, N3, Max 1HFL 6/2025 Rec: 1st HFL AI 1/2025 Non FD Gaud HFL sy VI		

Recommendations:	Remitted vs Benign
7/2025 Recy: local prog	Dr. Ali Radaly Canal
Redness,	↳ mostly benign
SM on 2/7/2025	for FU No Need for Biopsy

MDT OUTCOME & PLAN	PD of all of pr Bch mass
	2.2 x 1.5 (4 x 1.6 x 1.4)
	✓ 1/19/2025 for MDT

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Dr. Hossam	Dr. Hossam				
22/7/2025	22/7/2025				



Patient Name	: الأستاذ / سميرة عبدالسميع السيد ابراهيم	Request No.	: 13008466
Patient No.	: 248040501701243	Request Date	: 20/11/2024
Path No.	: 4499/24	Verification Date	: 07/12/2024
Sex	: Female	Financial Category	: اورام نادر للسلام - مبداءه
Age	: 76 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Left breast 12 O'clock mass.

Nature of Specimen: Tru-cut biopsy

Gross Examination:

- Four grayish white tissue cores up to 2 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Focal.
- * Perineural invasion: Not detected.
- * Angioinvasion: Not detected.
- * Microcalcifications: Not detected.

Diagnosis:

Left breast 12 O'clock mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II with focal DCIS.

- For further IHC (ER, PR, Her2 and Ki67).

Signature

Prof. Dr. Lobna Omar

Dr. Menna Nabil

Verified by :

Dr. Heba Hasan

Printing By : marwa mohamed youss

Printing Date : 07/12/2024 10:37:53

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misor El Qadema - Cairo, Egypt

Patient Name الأستاذ / سميرة عبدالمصعب السيد ابراهيم
Patient No. 248040501701243
Path No. 4499/24
Sex Female
Age 76 Y
Referring Doc.

Request No. 13008466
Request Date 20/11/2024
Verification Date 07/12/2024
Financial Category لورام دار السلام - مبالغه
Department Out Patient
Registered by marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within > 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Prof. Dr. Amal Hareedy
Dr. Menna Nabil

Verified by :
Dr. Heba Hasan

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Patient Name	: الأستاذة / سميرة عبدالسميع السيد ابراهيم	Request No.	: 13008466
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Sex	: Female	Financial Category	: اورام دار السلام - مبخرة
Age	: 76 Y	Department	: Out Patient
Referring Doc. :		IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.
Nature of Specimen: Tru-cut biopsy.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:

- ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 34% nuclear reaction of tumor cells.

Conclusion:

Left breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II.
IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67: Positive in 34% of tumor cells.

Comment:

- * Scoring system: "ER, PR"

Verified by :
Dr.Heba Hasan

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